



EVALUATION 5 — EXTREME ACUITY

Primary Diagnosis: Home IV TPN via Tunneled Central Line, Short Bowel Syndrome, Enterocutaneous Fistula, Wound VAC, Chronic Pain on Methadone, History of IVDU

PATIENT DEMOGRAPHICS

Field Detail

Patient Name Destiny Rayne Alvarez-Jackson

Date of Birth 07/03/1988

Age 36

Sex Female

Race/Ethnicity Hispanic/Latina (Mexican American) and Black/African American (multiracial)

Primary Language English

Interpreter Needed No; however, grandmother (in-home caregiver) is Spanish-speaking only — Spanish interpreter required for all caregiver teaching

Marital Status Single (in relationship — boyfriend not a reliable caregiver)

Medical Record Number CI-2025-00447

Address of Service 829 Mariposa Way, Apt 6, East Palo Alto, CA 94303

Cell Phone (650) 555-0179

County San Mateo

EMERGENCY CONTACTS

Priority Name Relationship Phone Lives with Patient?

1 Rosa Alvarez Grandmother (maternal) — primary caregiver (650) 555-0180 Yes

2 Marcus Jackson Father of patient's children (650) 555-0544 No — but has children on weekends

3 Gloria Alvarez-Torres Mother (650) 555-0831 No — but visits frequently

Children in Home

Jaylen Jackson, age 6 (kindergarten — Marcus has partial custody, with patient weekdays)

Aria Jackson, age 3 (home with patient/grandmother full-time)

Special Considerations: Two young children in home. Caregiver grandmother (age 74) is sole reliable adult support. Multiple infection risks from pediatric illnesses. Sharps and medications must be secured away from children. Wound VAC and TPN tubing — child safety concerns.

LIVING SITUATION

Field Detail

Lives With Grandmother Rosa (74, Spanish-only, mild HTN, arthritic but functional), 2 children (ages 6 and 3)

Residence Type Ground-floor 2BR apartment, rented

Stairs None

Pets None

DME in Home Wound VAC (KCI V.A.C. Ultra), IV pole, TPN infusion pump (Curlin 6000), tunneled Hickman catheter supplies, hospital bed (patient declined — using own queen bed), bedside commode (rarely used — patient ambulates to bathroom), digital scale, thermometer, BP monitor

PAYER INFORMATION

Priority Payer ID / Policy Number

Primary Medi-Cal Managed Care (Health Plan of San Mateo) HPSM-90412763B

Secondary N/A

Note: Medi-Cal managed care plan. Prior authorization required for most services. Home infusion services (TPN) authorized through Coram/CVS Specialty Infusion (contracted provider). Wound VAC rental authorized through KCI/3M.

PHYSICIAN INFORMATION

Role Name Specialty Phone

Attending / Certifying Dr. Jonathan Rivera General Surgery / Surgical Critical Care (650) 555-1215

GI Dr. Heather Kim Gastroenterology (650) 555-1320

Pain Management Dr. Sanjay Patel Pain Medicine / Addiction Medicine (650) 555-1445

Wound Care Dr. Angela Torres Wound Care / Hyperbaric Medicine (650) 555-1550

Psychiatry Dr. Nina Kovacs Psychiatry (650) 555-1780

Infectious Disease Dr. Christopher Wynn Infectious Disease (650) 555-1655

Nutrition Support Dr. Priya Sharma Clinical Nutrition / TPN Management (650) 555-1870

F2F Date: 06/12/2025 F2F By: Dr. Jonathan Rivera

REFERRAL / HOSPITALIZATION HISTORY

Event Facility Dates Reason

Hospital #3 Stanford Medical Center 05/15/2025 – 06/14/2025 Exploratory laparotomy with lysis of adhesions, small bowel resection (120 cm removed — ischemic bowel secondary to mesenteric venous thrombosis); complicated by anastomotic leak → reoperation 05/22 → enterocutaneous fistula formation; short bowel syndrome (estimated 90 cm functional small bowel remaining); Hickman catheter placement for TPN; wound VAC placement

Hospital #2 Stanford Medical Center 03/08/2025 – 03/22/2025 Small bowel obstruction, adhesive (conservative management); DVT left femoral vein (provoked — immobility + OCP use); IVC filter placed

Hospital #1 San Mateo Medical Center 01/15/2025 – 01/22/2025 Small bowel obstruction, adhesive (resolved with NG decompression)

Prior Surgical History Various Multiple dates Open appendectomy with perforation and peritonitis (2016); exploratory laparotomy with adhesiolysis (2019); C-section x2 (2018, 2021); cholecystectomy (2022) — extensive surgical abdomen with dense adhesions

Start of Care Date: 06/15/2025

DIAGNOSES (extremely complex — 24 active diagnoses)

Priority ICD-10 Diagnosis

Primary K91.2 Postsurgical malabsorption, not elsewhere classified (short bowel syndrome — 90 cm functional SB)
Surgical K63.2 Fistula of intestine (enterocutaneous fistula — right lower quadrant, draining 400-600 mL/day)
Surgical T81.4XXA Infection following a procedure, not elsewhere classified (intra-abdominal abscess, drained; currently on IV antibiotics)
Active Z93.4 Other artificial openings of gastrointestinal tract status (note: patient does not have an ostomy, but ECF functions similarly)
Active E44.1 Mild protein-calorie malnutrition (albumin 2.4, prealbumin 12 at discharge)
Active E86.0 Dehydration (ongoing — high-output ECF; requires IV fluid supplementation)
Active E87.6 Hypokalemia (chronic — GI losses; requires aggressive IV and oral supplementation)
Active E83.42 Hypomagnesemia (chronic — GI losses)
Active E87.1 Hyponatremia (Na 132 at discharge)
Active Z99.11 Dependence on parenteral nutrition (TPN via Hickman — 7 days/week, 14 hr/day cycling)
Active Z45.2 Encounter for adjustment and management of vascular access device (Hickman catheter)
Active I82.412 Acute embolism and thrombosis of left femoral vein (recent — on anticoagulation; IVC filter in place)
Active Z79.01 Long-term (current) use of anticoagulants (enoxaparin, transitioning to warfarin)
Active D62 Acute posthemorrhagic anemia (Hgb 8.1 at discharge; iron infusion completed inpatient)
Active G89.29 Other chronic pain (chronic abdominal pain — adhesive disease, ECF site, surgical)
Active F11.21 Opioid use disorder, in sustained remission (history of IV heroin use 2014-2017; methadone maintenance since 2017)
Active Z79.891 Long-term (current) use of opiate analgesic — methadone (dual purpose: OUD maintenance + pain)
Active F33.1 Major depressive disorder, recurrent, moderate
Active F43.10 Post-traumatic stress disorder, unspecified (childhood trauma; medical PTSD from repeated surgeries/hospitalizations)
Active F41.1 Generalized anxiety disorder
Active L03.311 Cellulitis of abdominal wall (resolving — periwound; on antibiotics)
Active N18.3 Chronic kidney disease, stage 3a (GFR 48 — AKI in hospital, partially recovered)
Active I10 Essential hypertension
Active K58.9 Irritable bowel syndrome (pre-existing — now complicated by short bowel)

ALLERGIES

Allergen Reaction Severity

Cefazolin Anaphylaxis (intraoperative — 2016) LIFE-THREATENING
Amoxicillin Diffuse urticaria, throat tightness Severe
Ciprofloxacin QT prolongation (documented on telemetry 2025) Severe
Ibuprofen / NSAIDs GI bleeding (2019) Moderate
Adhesive tape Blistering, skin tears Moderate
Eggs Nausea Mild — BUT relevant to TPN formulation (lipid emulsion) — TPN lipid source confirmed egg-free by pharmacy
CURRENT MEDICATIONS (27 medications — highest-complexity regimen)

#	Medication	Dose	Route	Frequency	Purpose	High-Risk / Critical	Notes
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1 TPN (Parenteral Nutrition) Custom formula (per nutrition support RD/MD) IV via Hickman Continuous 14 hr/day (6 PM – 8 AM cycling) Primary nutrition; SBS Pharmacy mixes; delivered weekly by Coram. Contains dextrose, amino acids, lipids, electrolytes, vitamins, trace elements. Patient/grandmother trained on pump setup/disconnect. Monitor glucose during infusion (hyperglycemia) and after disconnect (hypoglycemia).

2 Lipid emulsion 20% (Intralipid) Per TPN formula IV (mixed in TPN or separate) 3x/week Essential fatty acids Egg allergy — verified egg-free lipid source (SMOFlipid)

3 Normal Saline 0.9% 1000 mL IV via Hickman Daily (separate from TPN — run over 4 hrs during daytime) Volume replacement (ECF losses) Adjust based on I&O and electrolytes

4 Potassium chloride 40 mEq IV (in NS bag) Daily Hypokalemia replacement Monitor K+ — life-threatening if hypo or hyper

5 Magnesium sulfate 2 g IV 3x/week (M-W-F) Hypomagnesemia Monitor Mg levels weekly

6 Ertapenem (Invanz) 1 g IV Daily x 10 more days (through 06/25/2025) Intra-abdominal infection (polymicrobial) Cannot use cephalosporins (allergy) or ciprofloxacin (QT). Infuse over 30 min.

7 Metronidazole (Flagyl) 500 mg IV TID x 10 more days (through 06/25/2025) Anaerobic coverage (intra-abdominal) No alcohol — coordinate with addiction history. IV preferred over PO (absorption unreliable in SBS).

8 Methadone 80 mg PO Daily (split: 40 mg AM, 40 mg PM) OUD maintenance + chronic pain CONTROLLED SUBSTANCE — HIGH ALERT. Dispensed by methadone clinic (daily observed dosing transitioned to 2-week take-home during hospitalization per clinic exception). Home supply: 14-day take-home doses in lockbox. Monitor for diversion, misuse, sedation, respiratory depression. QTc monitoring (methadone prolongs QT).

9 Enoxaparin (Lovenox) 60 mg SubQ BID DVT treatment (left femoral) + IVC filter Transitioning to warfarin once INR therapeutic x 2 consecutive draws. Monitor anti-Xa if needed.

10 Warfarin (Coumadin) 5 mg (titrating) PO Daily (PM) Anticoagulation (bridge from enoxaparin) INR monitoring 2-3x/week during titration. Drug interaction with metronidazole (increases INR) — monitor closely. Target INR 2.0-3.0.

11 Loperamide (Imodium) 4 mg PO QID (before meals + HS) Reduce GI transit time / ECF output Max 16 mg/day; monitor for ileus

12 Cholestyramine (Questran) 4 g PO BID (between meals) Bile acid malabsorption (SBS) Give 1 hr before or 4 hrs after other oral medications — binds drugs

13 Omeprazole 40 mg PO BID Gastric hypersecretion (SBS-related)

14 Ondansetron (Zofran) 4 mg PO/SL Q8H PRN nausea Nausea QT prolongation risk — already on methadone (additive QT risk); limited use

15 Pancrelipase (Creon) 36,000 units PO With meals and snacks Pancreatic enzyme supplementation (malabsorption) Swallow whole — do not crush

16 Sertraline 100 mg PO Daily AM Depression / PTSD

17 Hydroxyzine (Vistaril) 25 mg PO TID PRN anxiety Anxiety (non-benzo — chosen due to OUD history) Sedation risk — additive with methadone

18 Trazodone 50 mg PO HS PRN insomnia Insomnia

19 Multivitamin (liquid) 15 mL PO Daily Nutritional deficiency Liquid formulation — better absorption in SBS

20 Vitamin B12 1000 mcg SubQ Monthly (1st of month) B12 deficiency (terminal ileum resected)

21 Folic acid 1 mg PO Daily Folate replacement

22 Vitamin D3 50,000 IU PO Weekly Vitamin D deficiency (malabsorption)

23 Zinc sulfate 220 mg PO Daily Zinc deficiency (high-output fistula)

24 Selenium 200 mcg PO Daily Selenium deficiency (SBS)

25 Iron sucrose (Venofer) 200 mg IV Weekly x 5 doses (then reassess) Iron deficiency anemia Infuse over 15 min.

26 Acetaminophen 650 mg PO Q6H PRN (max 2600 mg/day — reduced for liver health) Pain adjunct

27 Wound VAC supplies Per wound care protocol Topical/device Continuous -125 mmHg ECF fistula wound management KCI V.A.C. Ultra system — dressing change 3x/week per wound care MD orders

TOTAL MEDICATIONS: 27 IV Medications: 7 (TPN, lipids, NS, KCl, MgSO₄, ertapenem, metronidazole) Controlled Substances: Methadone 80 mg daily (take-home lockbox) Sharps in Home: Yes — enoxaparin syringes, B12 syringes, insulin syringes (for TPN glucose management if added) Hickman Catheter: Dual-lumen tunneled catheter, right subclavian vein, inserted 06/05/2025. Lumen 1: TPN/lipids/NS. Lumen 2: IV antibiotics/iron/magnesium.

WOUND / FISTULA ASSESSMENT

Enterocutaneous Fistula (ECF) — Right Lower Quadrant:

Parameter Finding

Location RLQ, 4 cm lateral to midline, 6 cm inferior to umbilicus

Fistula Orifice Approximately 1.5 cm diameter; visible bowel mucosa at orifice

Output 400-600 mL/day (high output — defined as >500 mL/day; fluctuates); output is bilious-green to brown liquid/semi-formed effluent

Peri wound Skin Severely excoriated and eroded in a 5 cm radius around fistula orifice due to enzymatic damage from enteric output.

Denudement, weeping, and pain (8/10 during dressing changes).

Current Management Wound VAC (KCI V.A.C. Ultra with V.E.R.A.F.L.O. Instill & Dwell) — black foam cut to fit around fistula; fistula orifice isolated with silicone catheter draining to separate collection bag; -125 mmHg continuous; instillation therapy with NS per wound care MD orders. Dressing change 3x/week (M-W-F) by skilled nurse.

Wound VAC Alarms Patient/grandmother educated on: low pressure alarm (leak), blockage alarm, battery alarm, canister full alarm. Written troubleshooting guide provided in English and Spanish.

Effluent Collection Separate fistula drain bag — patient/grandmother empty and record output Q shift (AM/PM).

Nutrition Correlation ECF output directly affects nutritional status — higher output = greater electrolyte and fluid losses = TPN adjustments needed. Weekly labs guide TPN reformulation.

Abdominal Surgical Wound

Parameter Finding

Location Midline laparotomy, extending from xiphoid to suprapubic area (reopened 05/22 — initially closed; dehiscence post-leak)

Total Length 28 cm

Status Open — healing by secondary intention. Wound VAC covers both the ECF peri wound area and the open portion of the midline wound.

Wound Bed 60% granulation, 25% slough, 15% fibrinous

Depth 2.5 cm at deepest point (midline, infraumbilical)

Undermining 1.5 cm at 3 o'clock and 9 o'clock positions

Drainage (non-fistula) Moderate serosanguineous

HOMEBOUND STATUS

CONFIRMED. Patient is severely debilitated. Connected to wound VAC 24/7 and TPN pump 14 hrs/day. Ambulates short distances only (20-30 feet) with significant fatigue and abdominal pain. Carrying wound VAC canister unit during ambulation is taxing. Leaving home requires disconnecting/managing multiple devices. Immunocompromised from malnutrition (albumin 2.4). Two young children requiring care create additional barriers to outings.

ADVANCE DIRECTIVES

Document Status

AHCD Yes — grandmother Rosa designated as agent (patient executed 06/2025 pre-discharge). Specific instruction: patient wants full aggressive treatment including ventilator; does NOT want prolonged CPR >30 minutes if no response.

POLST Yes — Full treatment.

SERVICES ORDERED

Service Discipline Frequency Duration

Skilled Nursing RN Daily x 4 weeks (IV meds, wound VAC, Hickman care, labs, TPN monitoring, teaching), then 3x/week ongoing Indefinite (minimum 60-day cert; expected ongoing need)

Physical Therapy PT 3x/week x 8 weeks 8 weeks

Occupational Therapy OT 2x/week x 4 weeks 4 weeks

Medical Social Work MSW 1x/week x 8 weeks 8 weeks

Home Health Aide HHA Daily (personal care, child safety monitoring, light housekeeping, meal support) Duration of need

Registered Dietitian (if ordered/available) RD 1x/week x 4 weeks, then biweekly 8 weeks

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

Goal Target

- 1 ECF output reduced to <200 mL/day (fistula maturation/reduction) 08/14/2025
- 2 Abdominal wound reduced in size by ≥30% (measurements) 08/14/2025
- 3 Periwound skin excoriation improved (intact periwound skin ≥50% circumference) 08/14/2025
- 4 No CLABSI or Hickman catheter complications 08/14/2025
- 5 Electrolytes stable (K+ 3.5-5.0, Mg 1.5-2.5, Na 135-145) for ≥70% of draws 08/14/2025
- 6 Albumin improved to ≥2.8 (from 2.4) 08/14/2025
- 7 Patient weight stable or gaining (target ≥125 lbs — currently 118) 08/14/2025
- 8 Intra-abdominal infection resolved (afebrile, WBC normal, antibiotics completed) 06/25/2025
- 9 Patient/grandmother demonstrate TPN pump setup, connect, disconnect, and Hickman care independently 07/15/2025
- 10 Patient/grandmother demonstrate wound VAC troubleshooting 07/15/2025
- 11 Anticoagulation stable (INR 2.0-3.0 on warfarin; enoxaparin discontinued) 07/15/2025
- 12 Zero substance relapse; methadone compliance verified 08/14/2025
- 13 Ambulate 150 feet without significant pain or fatigue 08/14/2025
- 14 PHQ-9 decreased to ≤10 (from current estimated 16) 08/14/2025
- 15 Child safety plan in place (sharp/meds/equipment secured) 06/22/2025
- 16 Zero falls, zero ED visits, zero readmissions 08/14/2025

PATIENT SIGNATURES — EVALUATION 5

Patient Signature: _____ Date: _____

Print Name: Destiny Rayne Alvarez-Jackson

Caregiver / Agent Signature: _____ Date: _____

Print Name: Rosa Alvarez

Interpreter Used: ☐ Yes — Spanish telephone interpreter (for grandmother teaching)

Clinician Signature: _____ Date: _____

Patient / Representative Signature

Date

RN Clinician Signature

Date / Time